

Client
Gurugram
 Pathkind Diagnostics Pvt. Ltd
 Plot No. 55-56, Udhayog Vihar Ph-IV, Gurugram - 122015

Processed By
 Pathkind Diagnostics Pvt. Ltd.
 Plot No. 55-56, Udhayog Vihar Ph-IV, Gurugram - 122015

Name	: Mr. HM05	Billing Date	: 07/07/2023 12:24:06
Age	: 35 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P. ID No.	: P1000100012780	Report Released on	: 14/07/2023 13:01:49
Accession No	: 10002304836	Barcode No.	: 10002304836-01
Referring Doctor	: Self		
Referred By	:	Ref no.	:

Report Status - Final			
Test Name	Result	Biological Ref. Interval	Unit

HAEMATOLOGY

Complete Blood Count (CBC)

Haemoglobin (Hb) Sample: Whole Blood EDTA Method: Photometric measurement	14.8	13.0 - 17.0	gm/dL
Total WBC Count / TLC Sample: Whole Blood EDTA Method: Impedance	7.2	4.0 - 10.0	thou/ μ L
RBC Count Sample: Whole Blood EDTA Method: Impedance	4.9	4.5 - 5.5	million/ μ L
PCV / Hematocrit Sample: Whole Blood EDTA Method: Impedance	42.9	40.0 - 50.0	%
MCV Sample: Whole Blood EDTA Method: Calculated	94.8	83.0 - 101.0	fL
MCH Sample: Whole Blood EDTA Method: Calculated	28.1	27.0 - 32.0	pg
MCHC Sample: Whole Blood EDTA Method: Calculated	32.4	31.5 - 34.5	g/dL
RDW (Red Cell Distribution Width) Sample: Whole Blood EDTA Method: Calculated	15.1	11.8 - 15.6	%
DLC (Differential Leucocyte Count) Method: Flowcytometry/Microscopy			
Neutrophils Sample: Whole Blood EDTA Method: VCS Technology & Microscopy	55	40 - 80	%

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10002304836 Mr. HM05

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Leukocytes	96	30 - 40	%
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Lymphocytes Sample: Whole Blood EDTA Method: VCS Technology & Microscopy	35	20 - 40	%
Eosinophils Sample: Whole Blood EDTA Method: VCS Technology & Microscopy	03	01 - 06	%
Monocytes Sample: Whole Blood EDTA Method: VCS Technology & Microscopy	07	02 - 10	%
Basophils Sample: Whole Blood EDTA Method: VCS Technology & Microscopy	00	00 - 02	%
Absolute Neutrophil Count Sample: Whole Blood EDTA	3960	2000 - 7000	/µL
Absolute Lymphocyte Count Sample: Whole Blood EDTA	2520	1000 - 3000	/µL
Absolute Eosinophil Count Sample: Whole Blood EDTA	216	20 - 500	/µL
Absolute Monocyte Count Sample: Whole Blood EDTA	504	200 - 1000	/µL
Absolute Basophil Count Sample: Whole Blood EDTA	00 L	20 - 100	/µL
Platelet Count Sample: Whole Blood EDTA Method: Impedance	190	150 - 410	thou/µL
MPV (Mean Platelet Volume) Sample: Whole Blood EDTA Method: Calculated	12.7 H	6.8 - 10.9	fL

Complete Blood Count (CBC)

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Clinical Significance :

CBC comprises of estimation of the cellular components of blood including RBCs, WBCs and Platelets. Mean corpuscular volume (MCV) is a measure of the size of the average RBC. MCH is a measure of the hemoglobin content of the average RBC and MCHC is the hemoglobin concentration per RBC. The red cell distribution width (RDW) is a measure of the degree of variation in RBC size (anisocytosis) and is helpful in distinguishing between some anemias. CBC examination is used as a screening tool to confirm a hematologic disorder, to establish or rule out a diagnosis, to detect an unsuspected hematologic disorder, or to monitor effects of radiation or chemotherapy. Abnormal results may be due to a primary disorder of the cell-producing organs or an underlying disease. Results should be interpreted in conjunction with the patient's clinical picture and appropriate additional testing performed.

** End of Report **



Dr. Aarti Khanna Nagpal

DNB (Pathology)

Senior Consultant

10002304836 Mr. HM05

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LIVER & KIDNEY PROFILE

BIOCHEMISTRY

Liver Function Test (LFT)

Sample : Serum

Bilirubin Total	0.49	0.00 - 1.20	mg/dL
Method : DIAZO			
Bilirubin Direct	0.23 H	0.00 - 0.20	mg/dL
Method : DIAZO			
Serum Bilirubin (Indirect)	0.26	0.00 - 0.90	mg/dL
Method : Calculated			
SGOT / AST	27.65	0.00 - 40.00	U/L
Method : UV Without PSP (Pyridoxal 5 Phosphatse)			
SGPT / ALT	21.50	0.00 - 41.00	U/L
Method : UV With PSP (Pyridoxal 5 Phosphatse)			
AST / ALT Ratio	1.29	-	Ratio
Method : Calculated			
Alkaline Phosphatase	97.00	40.00 - 129.00	U/L
Method : PNPP, AMP Buffer			
Total Protein	7.42	6.40 - 8.30	gm/dL
Method : BIURET			
Albumin	4.52	3.97 - 4.94	gm/dL
Method : Bromo Cresol Green (BCG)			
Globulin	2.90	1.90 - 3.70	gm/dL
Method : Calculated			
Albumin Globulin A/G Ratio	1.56	1.00 - 2.10	Ratio

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Method : Calculated

Kidney Function Test

Sample : Serum

Blood Urea Nitrogen	14.95	8.41 - 25.70	mg/dL
Method : Calculated			
Blood Urea	32.00	18.00 - 55.00	mg/dL
Method : LIASAGE IIV			



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Method: Calculated

Kidney Function Test

Sample: Serum

Blood Urea Nitrogen	14.95	8.41 - 25.70	mg/dL
---------------------	-------	--------------	-------

Method: Calculated

Blood Urea	32.00	18.00 - 55.00	mg/dL
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Method: Urease UV

Creatinine	1.15	0.70 - 1.30	mg/dL
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Method: Jaffe Kinetic IDMS

BUN Creatinine Ratio	13.00	10.00 - 20.00	Ratio
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Method: Calculated

Calcium	9.43	8.60 - 10.00	mg/dL
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Method: NM-BAPTA Complex

Uric Acid	5.90	3.60 - 8.20	mg/dL
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Method: Urlicase

Sodium	143.00	136.00 - 145.00	mmol/L
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Method: ISE-Indirect

Potassium	4.20	3.50 - 5.10	mmol/L
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Method: ISE-Indirect

Chloride	98.20	97.00 - 107.00	mmol/L
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Method: ISE-Indirect

Total Protein	7.42	6.40 - 8.30	gm/dL
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Method: BIURET

Albumin	4.52	3.97 - 4.94	gm/dL
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Method: Bromo Cresol Green(BCG)

Globulin	2.90	1.90 - 3.70	gm/dL
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Method: Calculated

Albumin Globulin A/G Ratio	1.56	1.00 - 2.10	Ratio
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Method: Calculated

Lactate Dehydrogenase (LDH)	208.00	135.00 - 225.00	U/L
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Method : Calculated

Lactate Dehydrogenase (LDH)	208.00	135.00 - 225.00	U/L
-----------------------------	--------	-----------------	-----

Sample : Serum

Method : Lactate Pyruvate UV

Gamma-Glutamyl Transferase (GGT)

Sample : Serum

Method : G- Glutamyl-Carboxy- Nitroanilide

Gamma-Glutamyl Transferase (GGT)	25.00	10.00 - 71.00	U/L
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CLINICAL PATHOLOGY

Urine Routine & Microscopic Examination

Sample : Urine, Random

Colour, Urine	Pale Yellow	Pale Yellow	---
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Method : Physical Examination

Appearance	Clear	Clear	---
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Method : Physical Examination

Specific Gravity	1.015	1.00 - 1.04	---
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Method : pKa change of pretreated polyelectrolytes

pH	5.00	4.70 - 7.50	---
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Method : Double Indicator principle

Glucose	Detected (+ + +)	Not Detected	---
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Method : Glucose oxidase/ peroxidase

Protein	Not Detected	Not Detected	---
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Method : Protein-Error of Indications Principle

Ketones	Not Detected	Not Detected	---
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Test Name	Result	Biological Ref. Interval	Unit
<i>Method: Sodium Nitroprusside Reaction</i>			
Blood	Not Detected	Not Detected	---
<i>Method: Peroxidase</i>			
Bilirubin	Not Detected	Not Detected	---
<i>Method: Diazo reaction</i>			
Urobilinogen	Normal	Normal	---
<i>Method: Ehrlich's Reaction</i>			
Nitrite	Not Detected	Not Detected	---
<i>Method: Nitrite Test</i>			
Pus Cells	1-2	0-5	/hpf
<i>Method: Microscopy</i>			
RBC	Not Detected	Not Detected	/hpf
<i>Method: Microscopy</i>			
Epithelial Cells	1-2	0-5	/hpf
<i>Method: Microscopy</i>			
Casts	Not Detected	Not Detected	---
<i>Method: Microscopy</i>			
Crystals	Not Detected	Not Detected	---
<i>Method: Microscopy</i>			
Bacteria	Not Detected	Not Detected	---
<i>Method: Microscopy</i>			
Remarks	Microscopic examination has been performed on urine sediment.		
<i>Method: Manual</i>			



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Interpretation

Lactate Dehydrogenase (LDH)

Lactate dehydrogenase (LD) levels are raised in megaloblastic anemia, untreated pernicious anemia, Hodgkin's disease, abdominal and lung cancers, severe shock, and hypoxia, myocardial infarction (MI), pulmonary infarction, pulmonary embolism, leukemia, hemolytic anemia, infectious mononucleosis, progressive muscular dystrophy, liver disease, and renal disease.

Kidney Function Test

Kidney function tests (KFT) are usually ordered when a patient has risk factors for kidney dysfunction such as high blood pressure (hypertension), diabetes, cardiovascular disease, obesity, elevated cholesterol or a family history of kidney disease. It may also be ordered when someone has signs and symptoms of kidney disease, though in early stage often no noticeable symptoms are observed. Kidney panel is useful for general health screening, screening patients at risk of developing kidney disease, management of patients with known kidney disease.

Liver Function Test (LFT)

Indications for Liver function assessment includes:

- Screen for liver infections, such as hepatitis
- Monitor the progression of a disease, such as viral or alcoholic hepatitis, and determine how well a treatment is working
- Measure the severity of a disease, particularly scarring of the liver (cirrhosis)
- Monitor possible side effects of medications

Urine Routine & Microscopic Examination



- Monitor the progression of a disease, such as viral or alcoholic hepatitis, and determine how well a treatment is working
- Measure the severity of a disease, particularly scarring of the liver (cirrhosis)
- Monitor possible side effects of medications

Urine Routine & Microscopic Examination

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Urine routine examination and microscopy comprises of a set of screening tests that can detect some common diseases like urinary tract infections, kidney disorders, liver problems, diabetes or other metabolic conditions. Physical characteristics (colour and appearance), chemical composition (glucose, protein, ketone, blood, bilirubin and urobilinogen) and microscopic content (pus cells, epithelial cells, RBCs, casts and crystals) are analyzed and reported.

** End of Report **

Authenticated By

Dr. Wallia Murshida Huda
MBBS MD

Dr. Aarti Nagpal
MBBS MD (Pathology)
HOD

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Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P. ID No.	: P1000100012601	Report Released on	: 14/07/2023 18:33:59
Accession No	: 10002304657	Barcode No.	: 10002304657-01
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BIOCHEMISTRY

Thyroid Profile Total

Total T3 (Trilodothyronine)

Sample: Serum
 Method: ECLIA

1.80

0.80 - 2.00

ng/mL

Total T4 (Thyroxine)

Sample: Serum
 Method: ECLIA

12.10

5.10 - 14.10

µg/dL

TSH 3rd Generation

Sample: Serum
 Method: ECLIA

4.600 H

0.270 - 4.200

µIU/mL

Total T3 (Trilodothyronine)

Clinical Significance:

Thyroid hormones, T3 and T4, which are secreted by the thyroid gland, regulate a number of developmental, metabolic, and neural activities throughout the body. The thyroid gland synthesizes 2 hormones - T3 and T4. T3 production in the thyroid gland constitutes approximately 20% of the total circulating T3, 80% being produced by peripheral conversion from T4. T3 is more potent biologically. Total T3 comprises of Free T3 and bound T3. Bound T3 remains bound to carrier proteins like thyroid-binding globulin, prealbumin, and albumin. Only the free forms are metabolically active. In hyperthyroidism, both T4 and T3 levels are usually elevated, but in some rare cases, only T3 elevation is also seen. In hypothyroidism T4 and T3 levels are both low. T3 levels are frequently low in sick or hospitalized euthyroid patients.

Total T4 (Thyroxine)

Clinical Significance:

Total T4 is synthesized in the thyroid gland. About 0.05% of circulating T4 is in the free or biologically active form. The remainder is bound to thyroxine-binding globulin (TBG), prealbumin, and albumin. High levels of T4 (and FT4) causes hyperthyroidism and low levels lead to hypothyroidism.

TSH 3rd Generation

10002304657 Mr. CL119



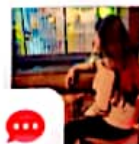
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Nicky Ranchi
Patient ko bhi Gali dete ho kya



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Clinical Significance:

TSH levels are elevated in primary hypothyroidism and low in primary hyperthyroidism. Evaluation of TSH is useful in the differential diagnosis of primary from secondary and tertiary hypothyroidism. In primary hypothyroidism, TSH levels are elevated, while secondary and tertiary hypothyroidism, TSH levels are low or normal. High TSH level in the presence of normal FT4 is subclinical hypothyroidism and low TSH with normal FT4 is called subclinical hyperthyroidism. Sick, hospitalized patients may have falsely low or transiently elevated TSH. Significant diurnal variation is also seen in TSH levels.

Guidelines for TSH levels in pregnancy, as per American Thyroid Association, are as follows:

TRIMESTER	BIOLOGICAL REFERENCE INTERVAL	UNIT
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Name	: Mr. CL119	Billing Date	: 07/07/2023 12:17:54
Age	: 35 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P. ID No.	: P1000100012601	Report Released on	: 14/07/2023 18:33:59
Accession No	: 10002304657	Barcode No.	: 10002304657-01
Referring Doctor	: Self	Ref no.	:
Referred By	:		

Report Status - Final			
Test Name	Result	Biological Ref. Interval	Unit
BIOCHEMISTRY			
Thyroid Profile Total			
Total T3 (Triiodothyronine)	1.80	0.80 - 2.00	ng/mL
Sample Serum Method: ECLIA			
Total T4 (Thyroxine)	12.10	5.10 - 14.10	µg/dL
Sample Serum Method: ECLIA			
TSH 3rd Generation	4.600 H	0.270 - 4.200	µIU/mL
Sample Serum Method: ECLIA			

Total T3 (Triiodothyronine)

Clinical Significance:
 Thyroid hormones, T3 and T4, which are secreted by the thyroid gland, regulate a number of developmental, metabolic, and neural activities throughout the body. The thyroid gland synthesizes 2 hormones - T3 and T4. T3 production in the thyroid gland constitutes approximately 20% of the total circulating T3, 80% being produced by peripheral conversion from T4. T3 is more potent biologically. Total T3 comprises of Free T3 and bound T3. Bound T3 remains bound to carrier proteins like thyroid-binding globulin, prealbumin, and albumin. Only the free forms are metabolically active. In hyperthyroidism, both T4 and T3 levels are usually elevated, but in some rare cases, only T3 elevation is also seen. In hypothyroidism T4 and T3 levels are both low. T3 levels are frequently low in sick or hospitalized euthyroid patients.

Total T4 (Thyroxine)

Clinical Significance:
 Total T4 is synthesized in the thyroid gland. About 0.05% of circulating T4 is in the free or biologically active form. The remainder is bound to thyroxine-binding globulin (TBG), prealbumin, and albumin. High levels of T4 (and FT4) causes hyperthyroidism and low levels lead to hypothyroidism.

TSH 3rd Generation

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10002304657 Mr. CL119

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 Plot No. 55-56, Udhog Vihar Ph-IV, Gurugram - 122015

Name	: Mr. CL119	Billing Date	: 07/07/2023 12:17:54
Age	: 35 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P. ID No.	: P1000100012601	Report Released on	: 14/07/2023 18:33:59
Accession No	: 10002304657	Barcode No.	: 10002304657-01
Referring Doctor	: Self	Ref no.	:
Referred By	:		

Report Status - Final			
Test Name	Result	Biological Ref. Interval	Unit
Clinical Significance:			
TSH levels are elevated in primary hypothyroidism and low in primary hyperthyroidism. Evaluation of TSH is useful in the differential diagnosis of primary from secondary and tertiary hypothyroidism. In primary hypothyroidism, TSH levels are elevated, while secondary and tertiary hypothyroidism, TSH levels are low or normal. High TSH level in the presence of normal FT4 is subclinical hypothyroidism and low TSH with normal FT4 is called subclinical hyperthyroidism. Sick, hospitalized patients may have falsely low or transiently elevated TSH. Significant diurnal variation is also seen in TSH levels.			

Guidelines for TSH levels in pregnancy, as per American Thyroid Association, are as follows:

REFERENCE INTERVAL	BIOLOGICAL REFERENCE INTERVAL	UNIT
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Name	: Mr. CL119	Billing Date	: 07/07/2023 12:17:54
Age	: 35 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P ID No.	: P1000100012601	Report Released on	: 14/07/2023 18:33:59
Accession No	: 10002304657	Barcode No.	: 10002304657-01
Referring Doctor	: Self		
Referred By	:	Ref no.	:

Report Status - Final

Test Name	Result	Biological Ref. Interval	Unit
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Clinical Significance :

TSH levels are elevated in primary hypothyroidism and low in primary hyperthyroidism. Evaluation of TSH is useful in the differential diagnosis of primary from secondary and tertiary hypothyroidism. In primary hypothyroidism, TSH levels are elevated, while secondary and tertiary hypothyroidism, TSH levels are low or normal. High TSH level in the presence of normal FT4 is subclinical hypothyroidism and low TSH with normal FT4 is called subclinical hyperthyroidism. Sick, hospitalized patients may have falsely low or transiently elevated TSH. Significant diurnal variation is also seen in TSH levels.

Guidelines for TSH levels in pregnancy, as per American Thyroid Association, are as follows:

PREGNANCY TRIMESTER	BIOLOGICAL REFERENCE INTERVAL	UNIT
FIRST TRIMESTER	0.100 - 2.500	µIU/mL
SECOND TRIMESTER	0.200 - 3.000	µIU/mL
THIRD TRIMESTER	0.300 - 3.000	µIU/mL

** End of Report **


Dr. Aarti Khanna Nagpal
 DNB (Pathology)
 Senior Consultant

10002304657 Mr. CL119



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Name	: Mr. HM12	Billing Date	: 07/07/2023 12:24:15
Age	: 35 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P. ID No.	: P1000100012787	Report Released on	: 14/07/2023 13:11:52
Accession No	: 10002304843	Barcode No.	: 10002304843-01
Referring Doctor	: Self		
Referred By	:	Ref no.	:

Report Status - Final

Test Name	Result	Biological Ref. Interval	Unit
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HAEMATOLOGY**Malaria Antigen Detection (Rapid)**

Method: Immunochromatography (pHRP II and pLDH antigen)

Plasmodium Vivax (PV)

Sample: Whole Blood EDTA

Detected

Not Detected

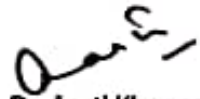
Plasmodium Falciparum (PF)

Sample: Whole Blood EDTA

Not Detected

Not Detected

** End of Report **



Dr. Aarti Khanna Nagpal

DNB (Pathology)

Senior Consultant

10002304843 Mr HM12

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Name	: Mr. HM12	Billing Date	: 07/07/2023 12:24:15
Age	: 35 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P. ID No.	: P1000100012787	Report Released on	: 14/07/2023 13:11:52
Accession No	: 10002304843	Barcode No.	: 10002304843-01
Referring Doctor	: Self	Ref no.	:
Referred By	:		

Report Status - Final

Test Name	Result	Biological Ref. Interval	Unit
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HAEMATOLOGY**Malaria Antigen Detection (Rapid)**

Method: Immunochromatography (pHRP II and pLDH antigen)

Plasmodium Vivax (PV)

Sample: Whole Blood EDTA

Detected

Not Detected

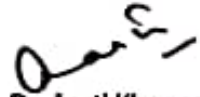
Plasmodium Falciparum (PF)

Sample: Whole Blood EDTA

Not Detected

Not Detected

** End of Report **



Dr. Aarti Khanna Nagpal

DNB (Pathology)

Senior Consultant

10002304843 Mr HM12

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Name	: Mr. HM13	Billing Date	: 07/07/2023 12:24:17
Age	: 35 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P. ID No.	: P1000100012788	Report Released on	: 14/07/2023 13:12:54
Accession No	: 10002304844	Barcode No.	: 10002304844-01
Referring Doctor	: Self	Ref no.	:
Referred By	:		

Report Status - Final

Test Name	Result	Biological Ref. Interval	Unit
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HAEMATOLOGY**Malarial Parasite (MP) Smear**

Method: Microscopy

Thin Smear

Sample: Whole Blood EDTA

Not Detected

Not Detected

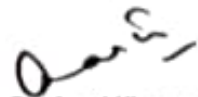
Thick Smear

Sample: Whole Blood EDTA

Not Detected

Not Detected

** End of Report **

**Dr. Aarti Khanna Nagpal**DNB (Pathology)
Senior Consultant

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Name :	Mrs. SE14	Billing Date :	07/07/2023 12:34:28
Age :	21 Yrs	Sample Collected on :	10/07/2023 10:01:31
Sex :	Female	Sample Received on :	10/07/2023 11:02:13
P ID No. :	P1000100013035	Report Released on :	18/07/2023 16:46:31
Accession No :	10002305091	Barcode No. :	10002305091-01
Referring Doctor :	Self	Ref no. :	
Referred By :			

Report Status - Final

Test Name	Result	Biological Ref. Interval	Unit
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SEROLOGY

WIDAL

Sample: Serum
 Method: agglutination method

Salmonella Typhi 'O'	< 1:80	< 1:80
Salmonella Typhi 'H'	1:320	< 1:80
Salmonella Paratyphi 'AH'	1:160	< 1:80
Salmonella Paratyphi 'BH'	1:160	< 1:80
Result :	Negative	

WIDAL

While the definitive diagnosis of typhoid fever depends on the isolation of *S typhi* from blood, stools, urine or other body fluids, the role of the Widal test had been to increase the index of suspicion for the presence of typhoid fever by demonstrating a positive agglutination during the aca convalescent period of infection with evidence of a four-fold rise of antibody titre.

Please note that the test suffers from serious cross-reactivity with other infectious agents, it may produce false-positive results, leading to an over-diagnosis of typhoid fever.

The IgM somatic O antibody appears first and represents the initial serologic response in acute typhoid fever, while the IgG flagella H antibody usually develops more slowly but persists for longer.

In an individual with no prior exposure to *S typhi* infection (either lack of active infection or absence of passive immunisation), a higher than 1:80 or 1:160 titre on an initial single test, usually indicates towards exposure to typhoid fever. However, even these single high value titres in an endemic area like India where repeated exposures to *S typhi* may have occurred, do not have any clinical relevance in the absence of a positive isolate of the causal organism OR demonstration of rising titers of antibodies by testing 2 or more serum samples 1-2 weeks apart.

Researchers from different parts of India have reported that in normally healthy blood donors, the baseline titre for antibodies to "O" and "H" antigens of

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Name :	Mrs. SE14	Billing Date :	07/07/2023 12:34:28
Age :	21 Yrs	Sample Collected on :	10/07/2023 10:01:31
Sex :	Female	Sample Received on :	10/07/2023 11:02:13
P ID No. :	P1000100013035	Report Released on :	18/07/2023 16:46:31
Accession No :	10002305091	Barcode No. :	10002305091-01
Referring Doctor :	Self	Ref no. :	
Referred By :			

Report Status - Final

Test Name	Result	Biological Ref. Interval	Unit
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Salmonella enterica serotype typhi was 1:40 and hence, based on the above results, it could be recommended to use a cutoff level of >1:80 for a single antibody test titre. Similarly, baseline titre for antibody to H antigen of Salmonella enterica serotype paratyphi A and paratyphi B was 1:80 and the cutoff level was >= 1:160 for a single antibody test titre.

** End of Report **

While the definitive diagnosis of typhoid fever depends on the isolation of *S. typhi* from blood, stools, urine or other body fluids, the role of the Widal test had been to increase the index of suspicion for the presence of typhoid fever by demonstrating a positive agglutination during the acute convalescent period of infection with evidence of a four-fold rise of antibody titre.

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Researchers from different parts of India have reported that in normally healthy blood donors, the baseline titre for antibodies to "O" and "H" antigens of

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Plot No. 55-56, Udyog Vihar Ph-IV, Gurugram - 122015

Name	: Mrs. SE14	Billing Date	: 07/07/2023 12:34:28
Age	: 21 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Female	Sample Received on	: 10/07/2023 11:02:13
P ID No.	: P1000100013035	Report Released on	: 18/07/2023 16:46:31
Accession No	: 10002305091	Barcode No.	: 10002305091-01
Referring Doctor	: Self		
Referred By	:	Ref no.	:

Report Status - Final

Test Name	Result	Biological Ref. Interval	Unit
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Salmonella enterica serotype typhi was 1:40 and hence, based on the above results, it could be recommended to use a cutoff level of >1:80 for a single antibody test titre. Similarly, baseline titre for antibody to H antigen of Salmonella enterica serotype paratyphi A and paratyphi B was 1:80 and the cutoff level was >= 1:160 for a single antibody test titre.

** End of Report **

Dr. Rahul Behl

MD

Consultant Microbiology

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Plot No. 55-56, Udhog Vihar Ph-IV, Gurugram - 122015

Name	: Mr. SE252	Billing Date	: 07/07/2023 12:36:57
Age	: 22 Yrs	Sample Collected on	: 10/07/2023 10:01:31
Sex	: Male	Sample Received on	: 10/07/2023 11:02:13
P. ID No.	: P1000100013137	Report Released on	: 17/07/2023 17:34:00
Accession No	: 10002305193	Barcode No.	: 10002305193-01
Referring Doctor	: Self		
Referred By	:	Ref no.	:

Report Status - Final

Test Name	Result	Biological Ref. Interval	Unit
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SEROLOGY
Typhidot IgG Rapid Card

Sample: Serum

Method: Immunochromatography

Detected

Typhidot IgM Rapid Card (Enterocheck)

Sample: Serum

Method: Immunochromatography

Detected

Typhidot IgM Rapid Card (Enterocheck)
Clinical Significance :

Typhidot IgM is a rapid card test used to detect enteric fever. This test is based on the presence of specific IgM antibodies to a specific antigen on S. typhi and becomes positive as early as in the first week of the fever.

Typhidot IgG & IgM, Rapid Card
Interpretation:

IgG & IgM Negative	Indicates absence of S. typhi antibodies.
IgM Positive	Indicative of current acute/ early infection with S. typhi.
IgG Positive	Indicative of previous S. typhi infection.
IgG & IgM Positive	Indicates acute typhoid fever (in the middle stage of S. typhi infection).

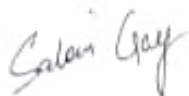
Clinical Significance:

This test is not a sole criterion for the diagnosis and these results should always be correlated with the clinical symptoms, and other laboratory investigations.

High titer Rheumatoid factor may result in a false positive reaction.

Cross reactivity may be observed with Salmonella paratyphi infection.

** End of Report **


Dr. Saloni Garg

MD

Consultant Microbiology



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Gurugram, Plot No. 55-56, Udhog Vihar Ph-IV, - 122015

Contact No. -7500075111

Name : Mr. DUMMY SE88
Age/Gender : 30 Yrs/Male
P. ID No. : 10002024419323
Accession No : 100020244190026
Referring Doctor :
Referred By : Self

Billing Date : 19/04/2024 03:00:02 PM
Sample Collected on : 19/04/2024 03:00:03 PM
Sample Received on : 23/04/2024 10:19:13 AM
Report Released on : 23/04/2024 10:19:37 AM

Report Status -Final

Test Name	Result	Biological Ref. Interval	Unit
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ELISA

Dengue Ns1 Antigen

Sample : Serum

Method : ELISA

Dengue NS1 Antigen Test, Elisa	0.88	Negative <0.8 Equivocal 0.8- <1.1 Positive >=1.1	Index
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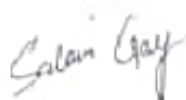
Interpretation

Dengue Ns1 Antigen

"The Dengue NS1 antigen test is used to detect the presence NS1 antigen of Dengue virus and is consistent with acute phase infection. The NS1 antigen is typically detectable within 1 to 2 days following infection and up to 9 days following symptom onset. NS1 antigen may also be detectable during secondary dengue virus infection, but for a shorter duration of time (1-4 days following symptom onset). The absence of dengue NS1 antigen is consistent with the lack of acute phase infection. The NS1 antigen may be negative if specimen is collected immediately following dengue virus infection (<24-48 hours) and is rarely detectable following 9 to 10 days of symptoms."

** End of Report **

Authenticated By



Dr. Saloni Garg
CONSULTANT MICROBIOLOGIST



100020244190026



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Contact No. -7500075111

Name : Mr. DUMMY SE89 **Billing Date** : 19/04/2024 03:01:32 PM
Age/Gender : 30 Yrs/Male **Sample Collected on** : 19/04/2024 03:01:33 PM
P. ID No. : 10002024419324 **Sample Received on** : 23/04/2024 10:20:30 AM
Accession No : 100020244190027 **Report Released on** : 23/04/2024 10:20:56 AM
Referring Doctor :
Referred By : Self

Report Status -Final			
Test Name	Result	Biological Ref. Interval	Unit

ELISA

Dengue Virus IgG & IgM Antibodies

Sample : Serum
Method : ELISA

Dengue Virus, IgM Antibodies, Elisa	0.72	Negative <0.8 Equivocal 0.8- <1.1 Positive >=1.1	Index
Dengue Virus, IgG Antibodies, Elisa	0.01	Negative <0.8 Equivocal 0.8- <1.1 Positive >=1.1	Index

Interpretation

Dengue Virus IgG & IgM Antibodies

Negative	<0.9 Index	<0.9 Index - It is recommended to test after 7-14 days or screen for Dengue NS1 antigen, IgG antibody assays to rule out early/acute infection
Equivocal	0.9-1.1 Index	May be due to low levels of IgM antibodies during the early stage of infection/nonspecific binding reactions. Repeat the test with a second sample within a 7-10 of time, if clinically indicated.
Positive	>1.1 Index	Presumptive evidence that the patient has been recently exposed to or is currently infected

Clinical Significance

This test should not be used as the sole criterion for the diagnosis of Dengue infection. Negative results/ suspected infections should be tested with Dengue NS1 antigen and dengue IgG antibody detection.
Serological cross reactivity across the Flavivirus group is common that is between St. Louis encephalitis, Murray Valley encephalitis, Japanese encephalitis, West Nile and yellow fever viruses.

** End of Report **



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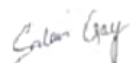
Booked For

Processed By
Gurugram, Plot No. 55-56, Udhog Vihar Ph-IV, - 122015
Contact No. -7500075111

Name : Mr. DUMMY SE89 **Billing Date** : 19/04/2024 03:01:32 PM
Age/Gender : 30 Yrs/Male **Sample Collected on** : 19/04/2024 03:01:33 PM
P. ID No. : 10002024419324 **Sample Received on** : 23/04/2024 10:20:30 AM
Accession No : 100020244190027 **Report Released on** : 23/04/2024 10:20:56 AM
Referring Doctor :
Referred By : Self

Report Status -Final			
Test Name	Result	Biological Ref. Interval	Unit

Authenticated By



Dr. Saloni Garg
CONSULTANT MICROBIOLOGIST

Equivocal 0.8-
<1.1

Positive ≥ 1.1

Interpretation

Dengue Virus IgG & IgM Antibodies

Negative	≤ 0.9 Index	≤ 0.9 Index - It is recommended to test after 7-14 days or screen for Dengue NS1 antigen. IgG antibody assays to rule out early/acute infection
Equivocal	0.9-1.1 Index	May be due to low levels of IgM antibodies during the early stage of infection/nonspecific binding reactions. Repeat the test with a second sample within a 7-10 of time, if clinically indicated
Positive	> 1.1 Index	Presumptive evidence that the patient has been recently exposed to or is currently infected

Clinical Significance:

This test should not be used as the sole criterion for the diagnosis of Dengue infection. Negative results/ suspected infections should be tested with Dengue NS1 antigen and dengue IgG antibody detection.

Serological cross reactivity across the Flavivirus group is common that is between St. Louis encephalitis, Murray Valley encephalitis, Japanese encephalitis, West Nile and yellow fever viruses.

** End of Report **



100020244190027

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PATHKIND DIAGNOSTICS PVT. LTD.
Plot No. 55-56, Udyog Vihar, Phase-4, Gurugram
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Page No. 1 of 2



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Processed By
Gurugram, Plot No. 55-56, Udyog Vihar Ph-IV, - 122015
Contact No. -7500075111

Name	: Mr. DUMMY SE89	Billing Date	: 19/04/2024 03:01:32 PM
Age/Gender	: 30 Yrs/Male	Sample Collected on	: 19/04/2024 03:01:33 PM
P. ID No.	: 10002024419324	Sample Received on	: 23/04/2024 10:20:30 AM
Accession No	: 100020244190027	Report Released on	: 23/04/2024 10:20:56 AM
Referring Doctor	:		
Referred By	: Self		

Report Status -Final

Test Name	Result	Biological Ref. Interval	Unit
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Authenticated By

Dr. Saloni Garg
CONSULTANT MICROBIOLOGIST

2/2



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Name	: Dummy	Billing Date	: 09/01/2024 07:36:50 PM
Age/Gender	:	Sample Collected on	: 09/01/2024 07:36:59 PM
P. ID No.	: Dummy	Sample Received on	: 10/01/2024 07:47:26 AM
Accession No	: Dummy	Report Released on	: 11/01/2024 12:24:00 PM
Referring Doctor	: Dummy		
Referred By	:		

Report Status -Final

SEROLOGY

Widal (Tube)

Element Name	Result	Ref.
Salmonella Typhi 'O'	< 1:80	< 1:80
Salmonella Typhi 'H'	< 1:80	< 1:80
Salmonella Paratyphi 'AH'	< 1:80	< 1:80
Salmonella Paratyphi 'BH'	< 1:80	< 1:80

Interpretation

WIDAL (Tube)

While the definitive diagnosis of typhoid fever depends on the isolation of *S typhi* from blood, stools, urine or other body fluids, the role of the Widal test had been to increase the index of suspicion for the presence of typhoid fever by demonstrating a positive agglutination during the acute and convalescent period of infection with evidence of a four-fold rise of antibody titre. In many developing countries, including India, the Widal test appears to be the only laboratory means employed in the diagnosis of typhoid fever among suspected patients. As the test suffers from serious cross-reactivity with other infectious agents, it may produce false-positive results, leading to an over-diagnosis of typhoid fever. The Widal test reaction involves the use of bacterial suspensions of *S typhi* and *S paratyphi* 'A' and 'B', treated to retain only the 'O' and 'H' antigens. These antigens are employed to detect corresponding antibodies in the serum of a patient suspected of having typhoid fever. The IgM somatic O antibody appears first and represents the initial serologic response in acute typhoid fever, while the IgG flagella H antibody usually develops more slowly but persists for longer.

In an individual with no prior exposure to *S typhi* infection (either lack of active infection or absence of passive immunisation), a higher than 1:80 or 1:160 titre on an initial single test, usually correlates fairly well with exposure to typhoid fever. However, even these single high value titres in an endemic area where repeated exposures to *S typhi* may have occurred, do not have any clinical relevance in the absence of a positive isolate of the causative organism.

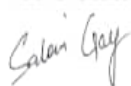
Researchers from different parts of India have reported that in normally healthy blood donors, the baseline titre for antibodies to "O" and "H" antigens of *Salmonella enterica* serotype typhi was 1:40 and hence, based on the above results, it could be recommended to use a cutoff level of $\geq 1:80$ for a single antibody test titre. Similarly, baseline titre for antibody to H antigen of *Salmonella enterica* serotype paratyphi A and paratyphi B was 1:80 and the cutoff level was $\geq 1:160$ for a single antibody test titre.

** End of Report **

Name	: Dummy	Billing Date	: 09/01/2024 07:36:50 PM
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** End of Report **

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